

CASE STUDY 1

We were approached by a 29 years old husband. He was very scared of his wife whom he thought was possessed. They were married for 7 months. It was an inter caste marriage. He loves his wife very much. He lives with his mother and brother. His father passed away 2 months after his marriage. About 10 days back, he had been for an off-sight team meeting and ever since he is back, his wife has been acting very weird. She seems to be possessed and he is avoiding eye contact. He has met several tantrics and done religious rituals to drive the negative energy. Nothing seems to work.

Name: Mr. A (Husband) , Mrs. A (wife)

City: Kolkata

Age: 29

Gender : Male

Problem: Husband of the view that his wife was possessed

Identifying Information

- **Names (pseudonyms):** Mr. A (husband), Mrs. A (wife)
- **Ages:** Mr. A – 29 years old; Mrs. A – 25 years old
- **Marital Status:** Married for 7 months.
- **Children:** none
- **Referral Source:** Referred by community health worker following reports of domestic disturbance and unusual behaviour attributed to supernatural causes.



Presenting problem:

Mr. A reports that his wife, Mrs. A, is “possessed by a spirit” and attributes her recent changes in behaviour including speaking in a different tone, mood swings, and social withdrawal, to supernatural causes. He has sought help from religious healers, tantrics and attempted rituals at home. Mrs. A appears distressed, confused, and reports feeling misunderstood and controlled.

History of Presenting Problem

According to Mr. A, symptoms began about 10 days back, when Mrs. A started becoming more withdrawn, often talking to herself, and occasionally appearing to stare into space. Mr. A notes episodes where she “talks like another person” and behaves “as if she doesn’t know who she is.” Instead of seeking psychiatric help, he turned to spiritual and religious interventions, believing the problem to be demonic possession.

Mrs. A reports feelings of extreme sadness, fatigue, disinterest in daily tasks, and occasional suicidal ideation. She says her husband refuses to believe her when she says she is depressed and instead accuses her of being “under the influence of a spirit.”

Mr. A is avoiding eye contact with her and ever since he returned from his team off-sight, her behaviour has become obvious and looks weird to him.

Family and Cultural Context

The couple belongs to a traditional community where spiritual and religious interpretations of mental illness are common. There is limited awareness and access to formal mental health services in their area.

Mr. A's belief system, shaped by his upbringing, views mental disturbances through a spiritual lens. Mrs. A, although religious herself, feels stigmatized and fears being labelled as "mad" or "cursed." or "possessed".

Mental Status Examination (MSE)

- **Appearance:** Mrs. A appears dishevelled, avoids eye contact.
- **Mood:** Depressed
- **Affect:** Blunted
- **Speech:** Soft, slow
- **Thought Process:** Coherent, but with intrusive thoughts of worthlessness.
- **Thought Content:** No delusions or hallucinations reported by Mrs. A; Mr. A insists she speaks in "other voices".
- **Cognition:** Alert and oriented
- **Insight:** Partial insight into her symptoms
- **Judgment:** Impaired, primarily due to emotional distress

Introduction

Cultural interpretations of mental illness, particularly in non-Western contexts, often include supernatural or spiritual explanations, such as spirit possession. While such beliefs can offer individuals and communities a framework for understanding suffering, they may also contribute to delays in psychiatric diagnosis and treatment (Koenig, 2009; Kirmayer et al., 2011). This case study illustrates these dynamics through the lens of a South Asian couple's experience with untreated psychiatric symptoms attributed to possession.



Psychological Formulation

- **Biopsychosocial model:**
 - **Biological:** Possible Major Depressive Disorder with dissociative features
 - **Psychological:** High emotional distress, possibly exacerbated by interpersonal dynamics and invalidation
 - **Social/Cultural:** Misinterpretation of symptoms due to cultural beliefs about possession; stigma around mental illness; power imbalance in the marriage

Systemic Issues: The husband's interpretation significantly influences how the wife is treated, including the decision to delay or avoid medical/psychiatric intervention.

Diagnosis (Provisional)

- **Major Depressive Disorder**, moderate, with possible **Dissociative Symptoms**
- Consider rule out: **Dissociative Identity Disorder** (less likely without more evidence), **Cultural Syndrome** (e.g., spirit possession syndrome under DSM-5's Cultural Formulation)

Goal to help them:

Treatment Plan

1. **Psychoeducation** for both spouses about mental health and depression
2. **Individual therapy** for Mrs. A (CBT or trauma-informed approach)
3. **Couples counseling** with a culturally competent therapist



4. **Cultural formulation interview (CFI)** to explore Mr. A's belief system and bridge understanding.
5. Involve **community leaders** or **religious figures** supportive of mental health treatment to increase buy-in
6. **Medical evaluation** to rule out neurological or metabolic causes.
7. **Safety plan** to address any risk of harm or coercion by the husband.

Prognosis

- Guarded to fair, depending on:
 - Willingness of Mr. A to engage with psychological understanding
 - Mrs. A's ability to access consistent therapy
 - Reduction in stigma and reinforcement of support networks

Ethical Considerations

- Ensuring informed consent from Mrs. A
- Monitoring for any form of emotional or physical abuse masked as spiritual intervention.
- Navigating cultural sensitivity without endorsing harmful practices



Outcome (3-Month Follow-Up)

- Mrs. B began responding to treatment, with reduced symptoms.
- Mr. A reported improved understanding and stopped seeking spiritual intervention exclusively.
- Ongoing therapy was recommended.

Discussion

This case illustrates how **cultural interpretations of mental illness**—such as belief in possession—can delay psychiatric care but can also serve as a bridge for engagement if approached with sensitivity. Collaborative care involving both medical and cultural frameworks often leads to better outcomes.

Conclusion

Mental health professionals must be equipped to recognize and work within cultural frameworks of illness. Collaboration with traditional healers, psychoeducation, and culturally appropriate interventions can bridge the gap between belief and biomedical understanding, leading to improved patient outcomes.

Verbatim between the anonymous and me as the counsellor

Mr. A : I lost my father a few months ago, and I just can't seem to move on. I'm trying to be strong for my family, but inside, I'm struggling to cope.

Me: I'm so sorry for your loss, Anonymous. It sounds like you've been carrying a lot of pain while also trying to be strong for others. Grief can be incredibly heavy, and it's normal to feel like you're not ready to move on yet. How have you been managing your feelings?



Mr. A: I don't know... I've just been keeping busy, trying not to think about it too much. But

when I'm alone, it all comes back, and I feel overwhelmed. I don't know how to deal with it. I try to take help of my family and off late I have been struggling with the same.

Me: It sounds like you've been pushing the grief down, but when it resurfaces, it becomes overwhelming. It's important to give yourself space to feel those emotions, even though they're difficult. How has been your experience when you discussed with your family?

Mr. A: Not that helpful. My wife off late has been behaving very weirdly. She comes across weird, avoiding eye contact. Making weird sounds and I am scared that she is possessed. I have been visiting tantric and other religious places and showing her to various priests, but not been able to address her problem.

Me: I understand that your wife's condition is not normal, In my view she has to go through some tests to assess her mental condition. Post the assessment only we will be able to diagnose what would be appropriate for her health and as a treatment for her.

Mr. A: I didn't think this could be a mental condition for my wife, rather thought that she has been possessed. I've just been avoiding it. Maybe I need to face it, but I don't know if I'm ready to let her go through all the assessment. But if this is going to help her, and provide a solution for her treatment, I am willing to go any mile.

Me: Lets start at a place and lets take one step at a time. We will walk you through the process, in detail.



Mr. A: Thanks for guiding me through, I would have ignored my wife's mental health condition, due to my ignore and beliefs on supernatural powers. Thank you for listening.

Recommendations:

1. Individual Cognitive-Behavioral Therapy (CBT)
2. Stress management and relaxation techniques. Gradual exposure to situations that trigger anxiety.
2. Psychiatric evaluation to assess the need for anti-anxiety medication
3. Development of a structured study schedule that includes mandatory breaks and self-care activities.
4. Training in mindfulness meditation to help manage racing thoughts and improve focus.
5. Gradual exposure to social situations and leisure activities to improve work-life balance.
6. Regular exercise routine to help manage stress and improve overall well-being.
7. Sleep hygiene education and strategies to improve sleep quality.
9. Group therapy or support groups coping strategies.
10. Family therapy sessions to address family dynamics
11. Regular check-ins to monitor suicidal ideation and develop a safety plan if needed.



Future Course of Action:

Schizophrenia is a complex disorder with no single known cause, but experts believe it arises from a combination of genetic, environmental, and brain-related factors. Specifically, research suggests that imbalances in brain chemistry, particularly in neurotransmitters like dopamine and glutamate, may play a role, along with potential structural abnormalities in the brain.

- Treatment typically involves a combination of medication (antipsychotics) and therapy, such as individual and group therapy, cognitive-behavioural therapy (CBT), and psychosocial rehabilitation.
- Medications help manage psychotic symptoms like hallucinations and delusions.
- Therapy helps individuals understand and manage their illness, develop coping skills, and improve social functioning.
- Psychosocial rehabilitation focuses on improving daily living skills, employment, and social connections.

Important Considerations:

- Schizophrenia is not a split personality or multiple personality disorder.
- Most people with schizophrenia are not dangerous and are more likely to be victims of crime.
- Early intervention is crucial for improving outcomes and preventing long-term disability.
- With appropriate treatment and support, many people with schizophrenia can live fulfilling lives.



Verbatim :

C – Counsellor

A – Husband

B - Wife

C: Thank you both for coming today. I understand this wasn't easy. Mr. A, would you like to start by telling me what brought you here?

Mr. A: (fidgeting) Yes... it's my wife. She's not the same. Something has... taken over her. I believe... she is possessed by a *jin*.

C: I see. Can you tell me what makes you think that?

Mr. A: She changes. She talks in a voice that's not hers. Low, angry. Sometimes she stares at the wall for hours. She doesn't talk to the children. She used to cook, clean, take care of everything. Now... she just lies there. Or screams at night.

C: That sounds very distressing for both of you. Mrs. B, can I ask—how have you been feeling?

Mrs. B: (softly) Tired. I... I don't know. Everything feels heavy. I don't want to get up.

Mr. A: (interrupts) But tell her about the voice. The one that came last week. You remember?

Mrs. B: (looks down, avoids eye contact) I was just... upset. Sometimes I feel like I'm outside myself. But it's still me. I'm not possessed. I'm just... tired of not being heard.

C: That's important to hear, thank you for sharing. You mentioned feeling "outside yourself." Can you say more about that?



Mrs. B: It's like... I'm watching myself. Like I'm not fully in my body. And sometimes I just want to scream. Or disappear.

C: Have you had thoughts of hurting yourself?

Mrs. B: (nods, quietly) Yes. Not every day. But... sometimes I think it would be easier.

Mr. A: (shocked) Why didn't you say this to me? You always say you're fine, then the next day you start mumbling or crying.

C: Mr. A, I understand this is very emotional for you. It's okay to feel overwhelmed. But let's try to focus on understanding what Mrs. B is going through, without jumping to conclusions just yet.

Mr. A: (quiet) I'm just scared. I don't know what else to do. The *maulvi saab* said it's a *jin*. We did *ruqyah*, gave her *tabeez*, but nothing works. I feel like she's slipping away.

C: Thank you for sharing that. Spiritual beliefs are a big part of many people's lives, and I want to respect that. What we're seeing here may also be signs of something treatable—like depression, or maybe stress-related symptoms that make her feel disconnected from herself.

Mr. A: You think it's medical?

C: It might be. Or psychological. But not in a way that means she's "crazy" or "cursed." Just something that needs healing, like any other illness. The way she's feeling—low energy, sadness, not wanting to live—those are not signs of possession. They're symptoms we see in people who are going through trauma, grief, or severe emotional pain.

Mrs. B: (whispers) No one ever said that to me before.

C: You're not alone. And there are ways we can help. Therapy, support, maybe medication if needed. But the first step is that you're here. That matters.



Mr. A: (sighs) I want to help her. But I don't understand all this... psychology. We never talked like this in our family.

C: That's okay. We'll go step by step. We can even bring in someone from your community who understands both your beliefs and mental health—if that would make things easier.

Mr. A: Like... the imam?

C: Possibly, yes. Only if you're comfortable. Our goal is to work together—with respect for your culture and your wife's healing.

Mrs. B: I just... I want to feel normal again.

C: And we'll help you get there.

 Notes from Clinician:

- **Mrs. B** presents with signs of **Major Depressive Disorder** with possible dissociative features.
- **Mr. A** is highly influenced by cultural/spiritual interpretations, but demonstrates **willingness to engage** if his beliefs are respected.
- Safety concerns exist (suicidal ideation), but no immediate intent expressed.
- Next steps: Psychoeducation, possible medication referral, individual therapy for Mrs. B, and culturally sensitive couples counseling.

Excellent choice — a **second session continuation** allows for deeper emotional exploration, therapeutic rapport-building, and early intervention strategies. In this session, we'll show:



- Progress (or tension) since the first meeting
 - Mrs. B beginning to open up more
 - Mr. A struggling with (but slowly shifting from) his possession belief
 - Clinician introducing basic psychoeducation gently
 - Cultural sensitivity being maintained throughout
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■ Verbatim Clinical Interview Transcript – Session 2

Setting: Community Mental Health Clinic

Participants:

- Clinician (C)
- Mr. A (Husband)
- Mrs. B (Wife)

Time Since Last Session: 1 week

C: Welcome back. I'm glad you both returned. How has the past week been?

Mr. A: (nods slowly) I'm not sure. Some days were better. Some... not so much. She slept more. Didn't scream though.

Mrs. B: (quietly) I tried to stay quiet. I didn't want to scare the children.

C: That's very thoughtful of you. Can you tell me more about how you were feeling during the week?



Mrs. B: (pauses) Numb. Mostly tired. Like I'm floating. My body feels like it's here, but my head isn't. I just lie down and wait for the day to pass.

Mr. A: (frowning) I kept watching her. Every time she went quiet, I kept thinking—what if the *jin* comes back?

C: I understand that thought must feel very real to you, Mr. A. Can I ask—have you noticed any patterns? Times when she seems more herself?

Mr. A: (thinks) Mornings. A little better in the morning. She even made chai two days ago.

Mrs. B: That day felt okay. I sat near the window. Sunlight helps, I think.

C: That's good insight. Some people with low mood find that light, routine, and nature help. Would it be okay if I explained a little about how the mind and body are connected?

Mr. A: (hesitant) As long as it's not against religion...

C: Absolutely. We're not replacing your beliefs. We're adding knowledge. Sometimes, deep sadness—grief, emotional exhaustion, even spiritual emptiness—can show up as strange behaviors. Not because of possession, but because the brain is overwhelmed. Like how fever means your body is fighting something.

Mr. A: (quietly) So... this might be like a fever... of the mind?

C: Exactly. A kind of emotional fever. And like physical illness, there are ways to help. Therapy is like a balm. And sometimes we use medicine—safe, approved medicine—not to change who someone is, but to help their mind settle so healing can begin.

Mrs. B: (looks up) Will it make me feel normal?

C: It can help bring balance. But the real strength is in you. You've already shown it—coming here, speaking your truth, surviving everything you've been through.



... maybe you were cursed. Or angry. I didn't know it was sadness... this deep.

Mrs. B: I kept trying to tell you, but... you only saw the *jin*. Not me.

Mr. A: (looks down) I'm sorry. I just didn't want to lose you.

C: These conversations are part of healing. Mrs. B, would you feel comfortable starting individual therapy—where we can talk more in private? And Mr. A, I can continue meeting with you too, to explore how to support her in a way that feels right for both of you.

Mrs. B: Yes. I think I need that.

Mr. A: Me too. I still have questions... but I'll listen.

C: That's a beautiful place to start.

 Clinician Notes:

- **Mrs. B** continues to exhibit depressive symptoms with dissociative elements.
- **Mr. A** shows increased **readiness to consider psychological explanations** alongside his spiritual beliefs.
- Rapport strengthened. Therapeutic alliance is forming.
- Introduced **basic psychoeducation** on depression and mind-body interaction using culturally resonant metaphors.
- Safety check done; no active suicidal intent expressed.
- Next steps: Start individual therapy (CBT-informed), explore medication referral. Offer culturally adapted couple sessions.



Setting: Community Mental Health Clinic

Participants:

- Clinician (C)
- Mrs. B (Wife)
- Mr. A (Husband)

Time Since Last Session: 1 week

Duration: 50 minutes

C: It's good to see you both again. How have the last few days been?

Mrs. B: (nods) I had one good day. I swept the front yard and even laughed at something my daughter said. But the next day I couldn't move. My body just felt... like a stone.

Mr. A: I saw it. I tried not to panic this time. I made breakfast, let her rest. It was hard, but I'm learning.

C: That's good to hear. Both of you are trying—differently, but together. That matters. Mrs. B, you mentioned you laughed—how did that feel?

Mrs. B: Strange... and warm. I forgot myself for a moment. Then the guilt came.

C: Guilt?

Mrs. B: For feeling okay. Like I'm not supposed to. Like I'm supposed to suffer for what happened.

C: You mean the miscarriage?



There's nothing there, but it feels heavy. I think... maybe I am cursed.

Mr. A: (visibly shaken) I didn't know you still dream like that...

C: Those dreams sound painful, but they're also messages—from your mind and body, trying to process loss. It wasn't your fault. And you're not cursed. You are grieving.

Mrs. B: (quietly) Then why does it feel like I'm disappearing?

C: That feeling—of fading, of disconnect—is common in trauma. Your brain goes into survival mode. One way to help is through therapy, and sometimes through medicine that can quiet those overwhelming signals.

Mr. A: (tense) Medicine? Like sleeping pills?

C: Not exactly. More like a vitamin for the brain. A small daily dose of an antidepressant can help stabilize mood and reduce the heaviness she's describing. Not forever—just enough to let her rebuild from the inside.

Mr. A: (pauses) But... will it change her? Make her someone else?

C: No. It's not about changing her. It's about helping her come back to herself.

Mrs. B: Will it stop the floating? The dreams?

C: It may reduce how often and how strong those experiences feel. We'll start slow, and we'll monitor how you feel together.

Mr. A: (sighs, rubs forehead) If it will bring her back... okay. But no strong pills. And nothing that makes her worse.

C: That's a very reasonable concern. We'll use the lowest effective dose, and I'll stay in touch with the doctor who prescribes it. You're part of this process too.



C: That's courageous. Thank you both for trusting this process. Next time, I'd like to speak with you each separately for part of the session—so you each have space to say what you need.

Mr. A: Okay. That's fair.

C: Healing isn't a straight road. But you've both taken steps, and I see the effort you're making. That effort is where the change begins.

Clinician Notes:

- **Mrs. B** beginning to verbalize **trauma, grief, and guilt** linked to miscarriage.
- Continues to exhibit **dissociative tendencies** and **low mood**, but has emerging moments of joy and emotional connection.
- **Mr. A** remains conflicted, but shows **growth in empathy**, willingness to compromise, and openness to psychiatric medication within limits.
- Psychoeducation on **grief, trauma, antidepressants** provided using **culturally resonant metaphors**.
- Medication referral initiated.
- Plan for **partial individual sessions** next time to deepen engagement.



Setting: Psychiatry Department, Community Hospital

Participants:

- **Dr. R (Psychiatrist, female, mid-40s)**
- **Mrs. B (Patient, 34-year-old homemaker)**
- **Mr. A (Husband, 38-year-old shopkeeper)**

Referral From: Community Mental Health Counsellor

Duration: 40 minutes

Dr. R: Good morning. Thank you for coming in today. I've read a little about your case from the counsellor you've been seeing. But I'd like to hear from you, in your own words. Mrs. B, can you tell me what's been happening?

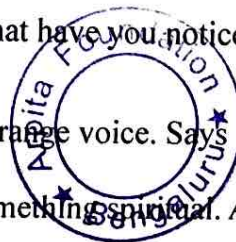
Mrs. B: (quietly) I feel tired all the time. I forget things. Some days I feel like I'm floating, not really in my body. I cry... even when nothing has happened.

Dr. R: That sounds very difficult. When did this start?

Mrs. B: After my miscarriage. Almost seven months ago. And then everything... slowed down. I stopped talking much. I stopped wanting to do anything. I don't want to live sometimes. But I also don't want to die. I just want to stop existing for a while.

Dr. R: I hear you. Thank you for being so open. Mr. A, what have you noticed?

Mr. A: (hesitant) She changes sometimes. She talks in a strange voice. Says things that don't sound like her. It scared me. At first I thought... maybe something spiritual. A *jin*. I took her to a religious healer.



Dr. R: That's understandable. Many families seek spiritual help, especially when symptoms are unfamiliar. Did anything improve with those treatments?

Mr. A: Not really. Sometimes she would get worse after the rituals.

Dr. R: You both have been through a lot. Mrs. B, when you say "floating," can you describe that more? Do you ever feel like someone else is taking over?

Mrs. B: (pauses) No. Not someone else. Just... a version of me that's not normal. I say things I don't mean. Sometimes I don't remember what I said. I hear my voice and it sounds far away.

Dr. R: Do you hear voices outside your head? Like someone talking to you when no one is there?

Mrs. B: No. Just thoughts. And dreams. Very loud dreams.

Dr. R: Do those dreams feel real? Do they affect how you feel when you wake up?

Mrs. B: Yes. I feel like I'm still in them. Sometimes I wake up crying. Or screaming.

Dr. R: Have you ever tried to harm yourself?

Mrs. B: No. But I've thought about not waking up.

Mr. A: (visibly distressed) She never told me that before.

Dr. R: These are symptoms of **Major Depressive Disorder**, possibly with some **dissociative features**—that sense of detachment you're feeling. Based on what you've described, this is not possession, but a psychological and emotional injury that needs healing.

Mr. A: (nervous) You're sure this isn't something spiritual?



Dr. R: I respect your beliefs deeply. And I believe spirituality can be part of healing. But what we're seeing here—low mood, disconnection, grief, sleep disturbance—these are known, treatable symptoms of depression. They can be helped with therapy and medication.

Mr. A: I'm just scared. Medicine can be strong. She's sensitive.

Dr. R: That's a good concern. We'll start low and go slow. A small dose of an SSRI—like **Sertraline**—can help her mood without sedating her. We'll monitor for side effects, and I'll stay in contact with your counsellor too.

Mrs. B: Will it help me feel... like myself again?

Dr. R: It won't change who you are. It will help your real self rise back up. Think of it like steadying the ground so you can walk again.

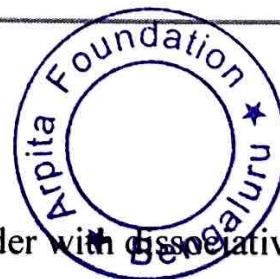
Mr. A: Can she still do her spiritual practices?

Dr. R: Absolutely. Many of my patients find prayer, Quranic recitation, and *duas* comforting. This is not an either/or situation. We're building a bridge between what you believe and what she needs medically.

Mrs. B: (nods slowly) I want to try. I don't want my children to be scared of me anymore.

Dr. R: You're incredibly brave. We'll start the medication today. I'll see you in two weeks for a follow-up, and you can call me earlier if there's any concern.

Psychiatric Notes (Dr. R):



- **Provisional Diagnosis:** Major Depressive Disorder with dissociative features
- **Medication Started:** Sertraline 25mg/day, titrate to 50mg in 1 week
- **Monitoring Plan:** Weekly check-ins via counsellor; medical follow-up in 2 weeks

- **Psychoeducation:** Provided on depression, grief, mind-body dissociation; spiritual framework acknowledged
- **Husband's understanding improving, but needs ongoing support and education.**

